Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

PROVISION IN REMOTE SITES

Provision of Anaesthesia in Radiology

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Key Points

- This is a guide to familiarise you with what to expect when anaesthetising for radiology. This does not cover every eventuality, please ask myself (Dr Boyer) if you need any advice.

General

Anaesthesia for radiology is provided in Interventional Radiology (IR) and CT. We do not have MRI compatible anaesthetic equipment, therefore GA and IV sedation cannot be provided for MRI.

Most of the patients coming through the unit are already in-patients or require the procedure urgently, in which case they will not have benefited from a pre-operative assessment clinic.

There are 2 cardiac arrest trolleys in radiology- one outside IR 08:30 –17:00 Mon to Fri (moved to ED xray out of hours) and one permanently in the holding bay of CT1.

If you need to access IR ring the doorbell, call 842165 or bleep the radiology nurse on 036.

If you need access to the CT scan room, either knock on the control room door or contact the on-call radiographer through switchboard.

Interventional Radiology

There is a GA list every Friday morning in IR. In addition IR may be required for emergency cases throughout the week.

There is a Dräger Atlan A350 anaesthetic machine, standardised airway trolley and C-Mac videolaryngoscope. There is a drug cupboard in the recovery area of IR which has all the commonly used GA drugs, as well as drugs required in an emergency. Refrigerated drugs and controlled drugs are kept in the IR store room. The radiology nurses have the keys for all these items.

All the equipment you need for an inhalational or TIVA GA and sedation is kept in IR. To provide a regional anaesthetic you will need to bring the kit with you.

In the recovery area of IR is wall-piped O_2 and suction. The radiology nurses recover the patients, however they are not fully trained recovery nurses, and do not get any specific training time for recovery, therefore you are expected to remain in IR until the patient is ready to be transferred to the ward. For complex patients, or in emergency cases you can ask to transfer the patient to main

recovery. However, this is not the routine arrangement and therefore may not be feasible depending on recovery's operational demands on that day.

CT

There are two CT scanners. CT1 is used preferentially for GA patients as there is more space, however make sure you know which CT scanner you are expecting to go to.

The most common reasons for anaesthetist attending CT scanner is either accompanying a patient requiring an emergency CT or patients who require an elective GA for CT scanning.

Accompanying patients for emergency CT

- This is an intrahospital transfer and should be treated as such, irrespective of where the transfer originates or how long it is expected to take. Ensure you have all the necessary equipment with you. In patients who are unintubated, and subsequently require intubation this should be undertaken in a suitable area (ED/ITU/anaesthetic room) not CT unless the patient is in cardiac arrest.

GA for planned CT

- This is done for patients who are unable to tolerate CT awake e.g. children, adults with learning difficulties
- These cases often require complex planning and, depending on the patient, it is usually best to anaesthetise in an anaesthetic room and transfer to CT. The other option is to take all the required equipment to CT and anaesthetise there but this should not be undertaken without extensive thought and planning.
- There is O_2 and suction in the "recovery" area of CT, however as you will need to recover the patient yourself it is best to take them to main recovery – you will need to agree this with recovery in advance. The patient is probably best woken up in an anaesthetic room.

Ionising Radiation

The radiographers are in charge of the IRMER regulations (ionising radiation) and you will be given protective clothing (gown, thyroid shield, X ray dense shield) as well as an indicator badge. It is useful for you to know that the penetration of X-ray radiation reduces a great deal the further you are from the source. It is vanishingly small at 2 metres from source.